



Sleep

DR. MARWA EZZAT

Excessive Daytime Sleepiness

- ✓ **Hypersomnia**
- ✓ **Narcolepsy**
- ✓ **Sleep apnea**
- ✓ **Insufficient sleep.**

Hypersomnia

❑ **Definition:** Hypersomnia refers to **conditions where the affected individual obtains sufficient sleep at night but still cannot stay awake during the day.**

❑ **Causes of hypersomnia**

1. Medical conditions, for example, CNS damage and certain kidney, liver disease.

2. Hypothyroidism.

Narcolepsy

- ❑ **Definition:** Narcolepsy is a disorder of **excessive daytime sleepiness**
- ❑ **Causes of narcolepsy**
 - **Lack of the chemical hypocretin in the area of the CNS that regulates sleep.**

Manifestation of narcolepsy

- Clients with narcolepsy have sleep attacks or excessive daytime sleepiness.
- Onset of symptoms tends to occur between ages 15 and 30, and symptom severity usually stabilizes within the first 5 years of onset.

Management of narcolepsy

- **CNS stimulants such as methylphenidate (Ritalin) have been used to reduce excessive daytime sleepiness.**

Sleep Apnea

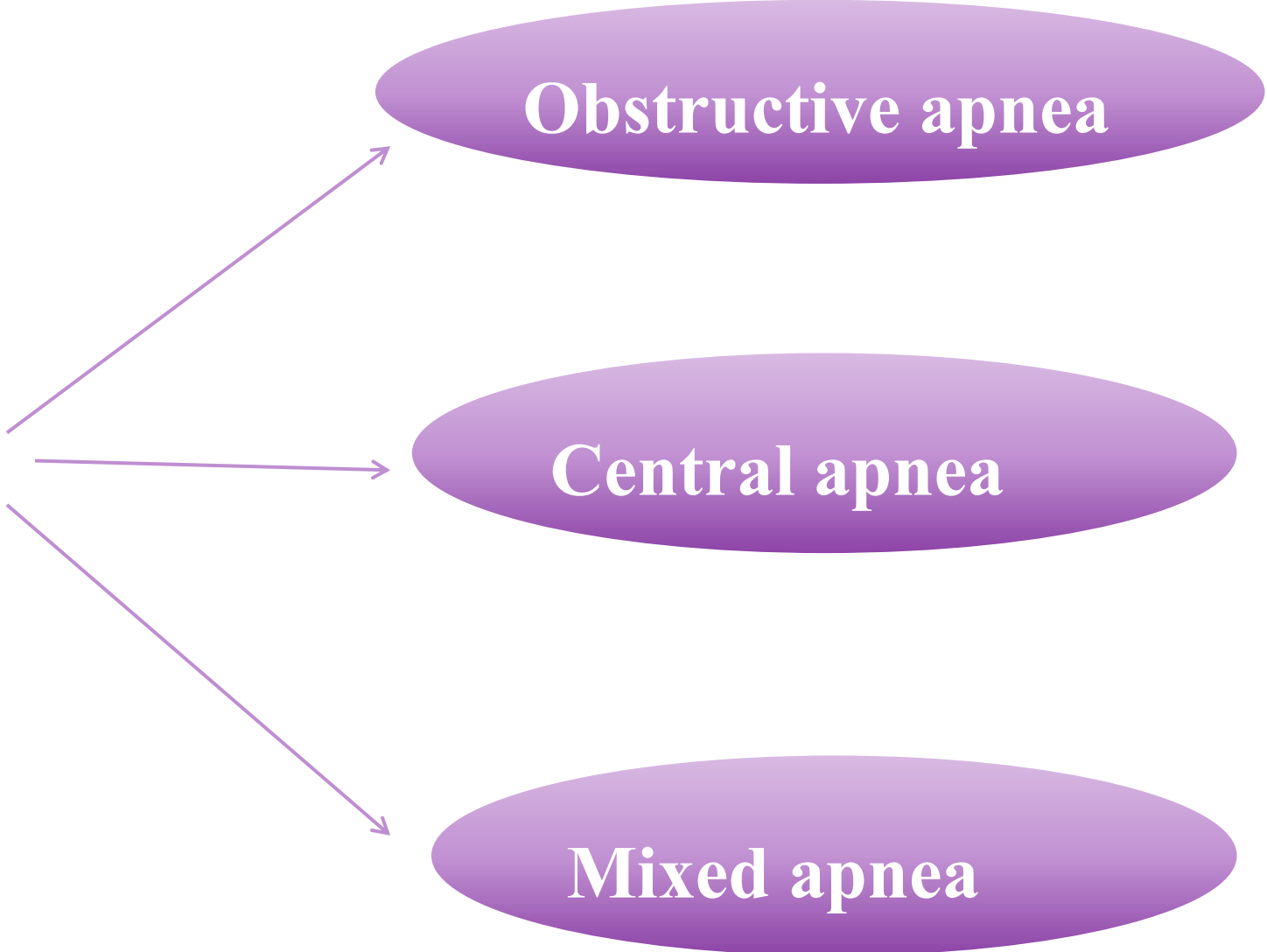
- Sleep apnea is characterized by frequent short breathing pauses during sleep.
- Although all individuals have occasional periods of apnea during sleep, more than five breathing pauses longer than 10 seconds per hour is considered abnormal.

- **Although** sleep apnea is **most frequently diagnosed in men and postmenopausal women**, it may occur during childhood.

The periods of apnea

- **Last from 10 seconds to 2 minutes, occur during REM or NREM sleep.**

Types of sleep apnea



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graph LR; A[Types of sleep apnea] --> B[Obstructive apnea]; A --> C[Central apnea]; A --> D[Mixed apnea];
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Obstructive apnea

Central apnea

Mixed apnea

Obstructive apnea

- Occurs when the structures of the pharynx or oral cavity block the flow of air.
- The individual continues to try to breathe; that is, the chest and abdominal muscles move.
- Enlarged tonsils, a deviated nasal septum, and obesity predispose the client to obstructive apnea.
- The episode of obstructive sleep apnea usually begins with snoring; thereafter, breathing ceases, followed by marked snoring as breathing resumes.

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- **Toward the end of each apneic episode, increased carbon dioxide levels in the blood cause the client to wake.**

Central apnea

- Central apnea is thought to involve **a defect in the respiratory center of the brain.**
- All actions involved in breathing, such as **chest movement and airflow cease.**

Mixed apnea

- **Mixed apnea is a combination of central apnea and obstructive apnea.**

Symptoms of sleep apnea

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- **Loud snoring**
 - **Frequent nocturnal awakenings,**
 - **Excessive daytime sleepiness.**
 - **Difficulties falling asleep at night**
 - **Morning headaches**
 - **Memory, cognitive problems, and irritability.**

Treatment for sleep apnea

Treatment for sleep apnea is directed at the cause of the apnea. For example:

- **Enlarged tonsils may be removed.**
- **Other surgical procedures, including laser removal of excess tissue in the pharynx, reduce or eliminate snoring and may be effective in relieving the apnea.**

Insufficient Sleep

- **Healthy individuals who obtain less sleep than they need will experience sleepiness and fatigue during the daytime hours.**

Symptoms of Insufficient Sleep

- **Attention and concentration deficits**
- **Distractibility**
- **Reduced motivation**
- **Fatigue**
- **Malaise**
- **Occasionally diplopia and dry mouth.**

Nursing management

Assessment

- **Sleep history**
- **Health history**
- **Physical examination**

Sleep History

Key questions to ask include the following:

- When do you usually go to sleep?
- When do you wake up?
- Do you nap? If so, when?
- If the client is a child, it is also important to ask about bedtime rituals.

- **Do you have any problems with your sleep?**
- **Has anyone ever told you that you snore loudly around a lot at night?**
- **Are you able to stay awake at work, when driving, or engaging in your usual activities?**
- **Do you take any prescribed medications, or herbal remedies to help you sleep? Or to stay awake?**

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- It is important to note that the **presence of a medical or psychiatric illness** (e.g., depression, Alzheimer's disease, or arthritis) may be contributing to the difficulty sleeping.

Physical Examination

- **Rarely are sleep abnormalities noted during the physical examination unless the client has obstructive sleep apnea or some other health problem.**
- **Common findings among clients with sleep apnea include an enlarged tonsils (in children), obesity (in adults)**

Sleep Diary

- **Time of (a) going to bed, (b) trying to fall asleep, (c) falling asleep (approximate time), (d) any instances of waking up and duration of these periods, (e) waking up in the morning, and (f) any naps and their duration**
- **Activities performed 2 to 3 hours before bedtime (type, duration, and time)**

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- **Consumption of caffeinated beverages and alcohol and amounts of those beverages**
 - **Any prescribed medications, and herbal remedies taken during the day**
 - **Bedtime rituals before sleep**

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- **Any difficulties remaining awake during the day and times when difficulties occurred**
 - **Any worries that the client believes may affect sleep**
 - **Factors that the client believes have a positive or negative effect on sleep.**

Nursing Diagnosis

- **Impaired sleep pattern**, related to overstimulation prior to bedtime

Various factors or etiologies may be included:

- Physical discomfort or pain; anxiety about actual or anticipated loss of a loved one, loss of a job, loss of life
- Due to serious disease process,
- Worry about a family member's behavior or illness

Planning

The major goal for clients with sleep disturbances is

- To maintain (or develop) a sleeping pattern that provides sufficient energy for daily activities.
- To enhancing the client's feeling of well-being or improving the quality and quantity of the client's sleep.

Implementing

- Interventions may include:

1. Providing comfort measures
2. Scheduling nursing care to provide for uninterrupted sleep periods
3. Teaching relaxation techniques.
4. Reducing environmental distractions
5. Support of bedtime rituals e.g. listening to music, reading, praying, massage, hot drink, & bathing
6. Conversing about accomplishments of the day or enjoyable events
7. Reducing environmental distractions من شرح النقطة دي فالشرائح القادمة مع ذكر امثلة
8. Enhancing sleep with medications مع ذكر امثلة من شرح النقطة دي فالشرائح القادمة
9. Health teaching about sleep habits مع ذكر امثلة من شرح النقطة دي فالشرائح القادمة

Health Teaching

- **Healthy individuals need to learn the importance of sleep in maintaining active and productive lifestyles.**
- **They need to learn**
 - (a) The conditions that promote sleep and those that interfere with sleep**
 - (b) Safe use of sleep medications**
 - (c) Effects of other prescribed medications on sleep**
 - (d) Effects of their disease states on sleep,**

Reducing Environmental Distractions

In Hospitals

- Close window curtains if streetlights shine through.
- Close curtains between clients in semiprivate and larger rooms.
- Providing extra blankets as needed.
- Reduce or eliminate overhead lighting; provide a night light at the bedside or in the bathroom.
- Lower the ringtone of nearby telephones.
- Before the client goes to bed, warm the bed with prewarmed bath blankets.
- Encourage the client to wear own clothing, such as pajamas, socks, sleeping cap or use extra blankets.
- Adhere to agency policy about times to turn off communal televisions or radios.

Enhancing Sleep with Medications

- Sleep medications often prescribed (as-needed) basis for clients include the sedative–hypnotics, which induce sleep, and antianxiety drugs or tranquilizers, which decrease anxiety and tension.

Evaluating

- The nurse judges whether client goals and outcomes have been achieved Or not.